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The case for the hospital business

Whether it beats leaving the money in treasury bills.

A hospital is worth building on one strategy and at one scale. On every other combination the capital does better in a government bond. This is the case for the one that works, and the evidence that the others do not.

October 2026 / Prepared for Dr Itunu Akinware

The money already has somewhere safe to be.

What the capital can earn today	Return	Real return after inflation
364-day treasury bill	15.89%	about 3%
FGN bonds	17 to 19%	2 to 2.5%
Inflation	15.91%	

This is not the Nigeria of two years ago. Fixed income now pays a positive real return, with no construction, no regulator, no clinical risk and no management. Any hospital has to beat 19 per cent before it has earned the right to exist.

A hospital clears it, but only one way.

23.4%

unlevered ten year return, 150 beds on the top strategy

29.1%

with 55 per cent debt at 14 per cent

	This hospital	The alternative
Ten year return	23.4% unlevered, 29.1% levered	17 to 19% in an FGN bond
What you own at the end	An asset worth NGN 165,491 M	Your money back
Pricing power	Tariffs reprice with inflation	Fixed coupon

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Ten of twelve ways to build it lose to a bond.

Unlevered ten year return	Low margin	Mid margin	High margin
30 beds	-10.3%	3.8%	14.2%
50 beds	-6.2%	6.7%	17.1%
100 beds	-1.7%	10.7%	21.3%
150 beds	0.6%	12.7%	23.4%

Ten year hold, exit at 8 times EBITDA. The bar is 17 to 19 per cent.

Only 2 of the twelve clear 19 per cent, and both are the top strategy at a hundred beds or more. This is not an argument that hospitals are good businesses. It is an argument that one kind of hospital is, and that the difference between them is entirely in the operating.

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Four decisions, none of them capital.

The decision	Loses to a bond	Beats it	Why it moves the number
Length of stay	4.5 days	2.8 days	The same beds carry 5,171 more admissions
Theatre throughput	1,900	6,200	The largest line, and the one the capital was spent on
Treating without admitting	2,400 day cases	14,000 day cases	Volume that never needs a bed
Diagnostics beyond your own patients	365,000 tests	1,200,000 tests	The highest margin line in the building

Revenue per occupied bed-day moves from NGN 306,754 to NGN 858,687 on identical bricks. Netcare, listed and at scale, runs about NGN 948,000. The top strategy is not heroic. It is ordinary competence measured against a real operator.

The demand is already leaving the country.

- **Nigerians spent about USD 685 million abroad on treatment last year.**
  Up from 643 million the year before. Oncology, cardiology, nephrology and orthopaedics drive roughly sixty per cent of it, and oncology alone is about forty per cent of cases.

- **That spend is addressable and it is dollar priced.**
  A patient who flies to India for a procedure is not price sensitive in naira. Capturing even a fraction of that at home is the single largest revenue pool in Nigerian healthcare.

- **The competition for it has just arrived.**
  The African Medical Centre of Excellence opened in Abuja with Afreximbank and King's College, 170 beds moving to 500, five theatres, three cath labs. Three of the four specialties driving outbound spend are now answered in Abuja. Lagos is not.

- **Fixed income will not stay here.**
  Today's positive real return is a function of a tight cycle. A hospital built now is priced against a bar that is unusually high, and it earns for thirty years.

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What it costs, and how much of it you can avoid.

Route	Capex, NGN M	Rent a year	What it costs you
Greenfield, new equipment	40,176	none	Best specification, longest programme, heaviest cheque
Greenfield, refurbished imaging	35,702	none	Saves roughly a fifth of the equipment bill. Older platform, shorter service life
Convert an existing building	27,683	none	No land, cheaper shell, worse geometry and higher mechanical cost
Lease and fit out	23,881	620	Lowest cheque. Rent becomes a permanent charge against EBITDA
Partner the imaging	22,396	none	Partner funds CT, MRI and cath for a share of the studies. Capex down, margin down

150 beds. Building at NGN 1,200,000 a square metre, equipment at published list converted at NGN 1,550 to the dollar.

The equipment is 51 per cent of the cheque and it is priced in dollars. That is the real exposure in a Nigerian hospital build, and it is the one the refurbished and partnered routes exist to manage.

Two years of nothing, then it compounds.



	NGN M	Share of everything the project returns
Operating cash, years 2 to 10	66,458	29%
The asset at the end	165,491	71%

Not a yield. An asset that reprices with inflation.

- **71 per cent of the return is the asset, not the cash.**
  A bond hands back the principal. A hospital at the end of ten years is worth NGN 165,491 million on an eight times multiple, and that multiple is itself a function of how well it was run.

- **The revenue reprices and the coupon does not.**
  Hospital tariffs move with inflation because the input costs do. A seventeen per cent bond bought today pays seventeen per cent in 2036 as well.

- **Leverage works here and nowhere else on the page.**
  Debt lifts the top strategy from 23.4 to 29.1 per cent. On the middle strategy it adds nothing, and on the low one it is ruinous. Borrow against competence, not against a building.

- **It is the platform the rest of the group sits on.**
  The theatre, the sterilising unit, the laboratory and the medical licence are what let an aesthetics, longevity and hair restoration business exist at all. That cluster earns NGN 4,220 million a year at 30 per cent and collects in cash on the day.

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The case rests on four things, and they are checkable.

What must hold	The number	How exposed it is
The top strategy is achievable	NGN 858,687 revenue per occupied bed-day	Low. Netcare runs about NGN 948,000 while listed and regulated
The margin is achievable	25.8% EBITDA	High. Netcare 18.6%, Life Healthcare 15.3%. This is the number to argue with
The build cost holds	NGN 1,200,000 a square metre	High. Judgement, not a quotation. Get it tendered before committing
The exit multiple holds	8 times EBITDA	Moderate. Transactions at this EBITDA size print 8 to 10 times

Two of the four are judgement rather than evidence. Tendering the build and stress testing the margin are a few weeks of work, and they are worth doing before the cheque rather than after it.

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The risks, and what answers each one.

Risk	What it costs	What answers it
The naira moves against equipment and reagents	NGN 660 M of EBITDA on a 20 per cent move	Refurbished platform, partnered imaging, dollar-linked tariffs
The operating strategy is not delivered	The whole case. 23.4% becomes 12.7%	This is the only risk that matters. Hire for it and measure it weekly
Key clinicians leave	NGN 950 M of lines sit on a handful of people	Equity, not salary. Build two deep in every anchored line
Payers slow down	Every 2 points of write-off is NGN 383 M	Cash and corporate mix, and the adjacent lines that collect same day
Construction overruns	Every month of delay is a month of the ramp	Fixed price contract, and a conversion route if greenfield slips

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What we are recommending

1. **Build it, at a hundred beds or more**
   Below that the capital cost per bed rises and the margin falls at the same time.
2. **Commit to the top strategy before the first brick**
   Short stay, heavy theatre, day case led, diagnostics sold beyond your own patients. The building does not create this. Management does.
3. **Tender the build and stress the margin first**
   The two numbers carrying the case are judgement. Two weeks of work converts them into evidence.
4. **Fund it with debt, but only once the strategy is signed**
   Leverage takes the top strategy to 29.1 per cent and the low one to minus twenty.
5. **Build the adjacent lines into the plan from day one**
   They need the hospital to exist, they earn 30 per cent, and they collect in cash while the hospital waits ninety days.